

HUSAM AHMED

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Hiring Manager

Health Technology and Systems Team
Canada's Drug Agency (CDA-AMC)

Re: Clinical Research Assistant (Temporary)

Dear Hiring Manager,

I'm applying for the Clinical Research Assistant role because the day-to-day work, screening literature, extracting data from studies, drafting reports, contributing to committee outputs, lines up closely with what I'm already doing in two other research environments. I'd like to bring those habits to health technology assessment and learn the systematic review methodology at the centre of the role.

At Stelis Environmental Solutions, I helped consolidate the team's SOP by working Health Canada GCDWQ, Ontario Regulations 169/03 and 170/03, and US EPA drinking-water rules side-by-side, mapping where each rule overlapped or diverged across jurisdictions and consolidating the requirements into one operating reference. The regulatory change log I maintain alongside it is change impact analysis in practice: when a rule shifts, I read the new text against the current SOP clause by clause, flag affected sections and downstream procedures, surface the compliance risk to project leads, and support the SOP revision and sign-off.

On the ColiMinder validation work, I support the team's pilot by reading peer-reviewed studies on enzymatic β -glucuronidase detection, noting the water matrices and conditions authors tested in, and comparing their reported sensitivity and false-positive rates against our pilot data. In Excel I align paired samples by site and timestamp (real-time readings against culture references that return 24 to 48 hours later), calculate concordance, and isolate false positives and false negatives to check for clustering by site, water matrix, or timing. Cross-jurisdictional Power BI heat maps from public Health Canada, US EPA, and provincial reports help frame prioritization. The output, comparison tables and briefs project leads use to weigh tradeoffs in speed, verification, and regulatory acceptance, sits in the same primary-source-to-decision space this role supports.

At IRCC's Operations Support Centre, my parallel role is investigative case processing under IRPA, reading partner-system traces and documenting reasoning so each file holds full context.

My BSc in Biology from the University of Ottawa is the foundation: microbiology coursework with structured sample handling, statistical analysis of lab datasets in Excel, and technical lab reports laying out methods, results, and limitations. I haven't done formal systematic review or meta-analysis work yet, and the chance to learn that under senior team members at CDA-AMC is one of the things that drew me to this role. The supporting habits, structured skepticism, careful reading, and attention to method and limitation, are already there.

A self-directed project I've been building (curious-brain) is hands-on practice in extracting specific mechanisms from peer-reviewed neuroscience papers and applying them in code. Each component traces to a specific paper: theta-gamma coupling from Lisman-Jensen 2008, predictive coding from Friston. Reading a study, isolating the testable mechanism, and translating it into a working system is the same primary-source-to-application motion behind evidence-based reviews.

I'm bilingual in English and Arabic with working knowledge of French, eligible for federal security clearance, and currently managing two simultaneous roles. I'd value the chance to bring this combination of primary-source reading, structured data work, and a builder's curiosity to the Health Technology and Systems team.

Thank you for your consideration.

Sincerely,

Husam Ahmed